Springfield General Hospital — Clinical Documentation Policy (FICTIONAL SAMPLE)

This is a fictional sample document for AI demonstration purposes only. It does not represent any real institution, patient, or policy. Springfield General Hospital is an invented facility.

Discharge summary requirements

Every discharge summary must be completed within 48 hours of discharge and include: admitting and discharge diagnoses, a hospital-course narrative in plain language, medication reconciliation listing all changes with reasons, pending laboratory results with the responsible follow-up provider, and scheduled follow-up appointments. Summaries for patients discharged to skilled nursing facilities must additionally include functional status and dietary requirements.

Prior authorization workflow

Prior authorization requests must cite the specific plan criteria being met, attach relevant clinical notes from the preceding 90 days, and document at least one guideline-concordant conservative treatment attempted, where clinically appropriate. Requests for advanced imaging (MRI, CT, PET) require documentation of the clinical question the study will answer. Urgent requests are flagged when a delay would jeopardize life, limb, or eyesight, and must be decided within 72 hours.

Documentation turnaround standards

Clinic visit notes must be signed within 72 hours. Operative notes require a brief note immediately post-procedure and a full note within 24 hours. Verbal orders must be countersigned within 48 hours. Departments falling below 90 percent on-time completion for two consecutive months receive documentation support resources.

Protected health information handling

Patient data may not be entered into external cloud services that are not covered by a business associate agreement. AI tools used for documentation assistance must run on hospital-controlled infrastructure or under an executed BAA. De-identified data used for quality improvement must meet Safe Harbor requirements: removal of all 18 HIPAA identifiers.

Coding and billing documentation

Evaluation and management levels must be supported by either medical decision making or total time. Time-based billing requires a statement of total time and a summary of activities. Split or shared visits must identify the substantive portion performed by the billing provider.